# One person can run the clinic office

The admin and front-desk day, from setup to closing, with automation supervising routine work.

### What this guide explains

This is the operating plan for a small clinic with one office operator. The person configures the clinic in the Admin workspace and runs patient-facing operations in the Receptionist workspace. The assistants take routine booking and information requests; the operator resolves exceptions. Clinical decisions stay with the doctor.

## The staffing model

| Work | Handled by Sahayak | Handled by the one operator |
|---|---|---|
| Routine patient enquiry | Replies from approved clinic facts and guides supported booking conversations. | Corrects settings and takes over requests that need a person. |
| Appointment | Offers real slots and saves a confirmed booking. | Resolves unusual scheduling, cancellation and rebooking cases. |
| Arrival and queue | Creates token/check-in state and live display when a patient self-checks in. | Helps walk-ins, no-shows, holds and people who need assistance. |
| Follow-through | Tracks tasks, check-ins and consent-based message workflows. | Watches failures, callbacks and unresolved cases; returns clinical issues to doctor. |

### Current login boundary

The application has separate Admin and Receptionist roles. One person can do both jobs with separate accounts, but the Admin account cannot use receptionist clinical/operational endpoints. A single combined login would require a deliberate role and permission change.

# 01 Set up the clinic once

| Step | What the operator enters or checks | Why it matters |
|---|---|---|
| 1. Clinic facts | Name, address, directions, phone, hours, holidays, average consultation time. | Patient answers and available slots depend on correct facts. |
| 2. Doctors | Create doctor profiles, schedules and slot duration; create each doctor login. | Patients are offered the right doctor and times; doctors see their own work. |
| 3. Office access | Create the receptionist login for the same office operator; keep credentials separate. | The front-desk queue and conversations remain role protected. |
| 4. Questions | Choose standard intake questions and English/Hindi answers to common clinic questions. | The assistant collects useful context and answers routine queries consistently. |
| 5. Channels | Configure WhatsApp, phone and other connections; inspect diagnostics and run real test messages/calls before claiming live readiness. | A saved connection is not proof of delivery. |
| 6. Automation | Describe a reminder/update, inspect the proposed trigger, audience and message, preview it, then activate if appropriate. | Nothing should run before review; consent and quiet-hour checks still apply. |
| 7. Patient check-ins | Review invitation settings, approved template, responsible care staff and delivery pause/enable state. | Post-visit outreach needs permission and a working channel. |

## Key rule for one-person scale

Put stable clinic facts and schedules in settings so the assistant can answer and book without the operator typing every response. Keep unusual cases in one visible work list.

# 02 Morning: open the office

- **Admin check:** confirm today’s hours, doctor availability, closed slots and channel status. Update a changed schedule before patients book against it.
- **Reception check:** open Today, inspect appointments, live queue, overnight conversations, urgent flags and pending callbacks.
- **Prepare exceptions:** identify patients whose booking changed, a doctor who is late, missing consent, an unresolved message or a failed send.
- **Leave routine flow on:** the standard appointment menu can answer enquiries and save confirmed bookings; the operator intervenes only when the flow needs help.

## A booking that arrives automatically

| Patient action | System action | Operator action |
|---|---|---|
| Asks to book | Shows clinic menu, visit type, real availability and asks for explicit YES. | No manual entry if the booking completes normally. |
| Confirms a slot | Saves the appointment to the clinic list. | Reviews only conflicting or unusual requests. |
| Requests a person or reports an urgent symptom | Routes to a staff conversation or raises an urgent alert. | Takes ownership immediately and involves the doctor or emergency pathway as required. |

### What one operator gains

Instead of handling every enquiry, the operator supervises the exceptions: walk-ins, confused patients, schedule conflicts, urgent flags and delivery problems. The clinic still needs human coverage while it is open.

# 03 During clinic: manage exceptions

## Arrivals and waiting room

- Point patients to the QR/check-in page; manually check in anyone who needs help or arrives without a booking.
- Watch tokens, queue order and statuses. Handle no-shows, held/skipped patients and practical waiting-room questions.
- When the doctor calls next, the live queue changes. Tell the doctor if an arrival or appointment needs manual attention.

## Messages, delays and cancellations

- Take over conversations that ask for a human or cannot be answered from configured clinic facts.
- When the doctor is late, wait for the doctor's impact review and confirmation. Then handle replies, rebooking and patients who cannot use the automated offer.
- For cancellations, check waiting-list eligibility, patient opt-in and the time-limited offer. Do not promise a slot until acceptance succeeds.
- Treat red-flag alerts as urgent. The assistant supplies a fixed emergency response, but staff must take ownership of the clinic response.

## Patient check-ins and follow-up

Watch the receptionist check-in list, callback owner and contact history. Record contact made, patient reply and next action. Refer clinical concerns to the doctor; a receptionist cannot close them by recording an operational callback alone.

**Priority order for a lone operator**

1. Urgent alerts and patients needing immediate help. 2. Doctor/queue interruptions. 3. Live human conversations and callbacks. 4. Delivery failures and waiting-list offers. 5. Routine configuration and reporting.

# 04 Closing and future automation

## End-of-day checklist

- Review unresolved conversations, urgent flags, no-shows and offers that expired or need an owner.
- Confirm tomorrow’s doctor schedule, holidays, opening hours and any planned changes.
- Check delivery status and planned messages. A queued or accepted message is not automatically a delivered message.
- Review feedback/callback items, privacy requests and clinic improvement tasks; assign the next action.
- Send the doctor clinical items that need review, and keep operational notes separate from medical decisions.

## What to automate next

| Priority | Automation | Release condition |
|---|---|---|
| 1 | Reliable live booking confirmation and reminders. | Real channel send/receive, consent, quiet hours, duplicate protection and delivery receipts. |
| 2 | Post-visit check-in invitation and patient reply matching. | Approved template, public HTTPS form, owner assignment, escalation and opt-out. |
| 3 | One operator dashboard and unified authorized login. | Explicit combined-role design, permission tests and audit trail. |
| 4 | Daily exception summary. | Use actual records and failures; measure operator time before claiming staff savings. |

**Current limits:** one person can operate both workspaces with separate logins. Real WhatsApp delivery still requires end-to-end verification for the configured clinic; WhatsApp Calling is not ready on the documented pilot number. Clinical advice and sign-off remain with the doctor.

Source basis: apps/web/app/admin/*, apps/web/app/console/*, apps/web/app/setup/page.tsx; docs/clinic-workspaces.md; docs/CLINIC_APPOINTMENT_FLOW.md; docs/patient-checkins-plan.md; docs/WHATSAPP_ASSISTED_SETUP.md.